

MEDICAL REPORT

Report ID: Recording 3

Date: 2025-11-06T10:07:07.573866

Patient Name:	N/A	Referring Dr.:	N/A
Age:	N/A	Modality:	Consultation
Sex:	N/A	Exam:	N/A
DOB:	N/A	Source File:	Recording 3.m4a

CLINICAL HISTORY:

The patient reports left knee pain persisting for more than a year, exacerbated by cold weather and exposure to cold water, alleviated with warmth. The pain is constant and was sudden in onset. There is associated morning stiffness lasting less than ten minutes, occurring in cold conditions. There is no swelling, giving way, or locking of the left knee. The patient denies any other joint issues or relevant back pain. No history of prior joint problems or specific past medical complaints related to this condition over the past two years. The family medical history reveals a female relative with a lymphatic condition affecting her knees, treated with radiation therapy, and a history of rheumatoid condition in another family member. The patient has hypothyroidism, taking 100 micrograms of thyroxine daily, and lives on the third floor of a building with an elevator. Occasional smoking is noted in social settings.

FINDINGS:

1. Left knee pain for over a year.

Chronicity suggests possible degenerative or inflammatory process, not acute injury.

2. Pain exacerbated by cold exposure.

Suggests inflammatory component possibly related to environmental triggers.

3. Sudden onset of knee pain.

Could indicate an acute event initially, although now presenting as chronic.

4. Morning stiffness less than ten minutes.

Short duration stiffness may suggest non-inflammatory arthritis.

5. No swelling or joint effusion.

Suggests absence of significant inflammatory process or synovitis.

6. Family history of lymphatic disease and rheumatoid conditions.

May indicate a genetic predisposition to joint or autoimmune conditions.

7. 100 micrograms of thyroxine daily for hypothyroidism.

Controlled hypothyroidism unlikely to directly contribute to joint symptoms but relevant for comprehensive care.

8. Occasional social smoking.

Social smoking may not directly affect the condition but is relevant for overall health assessment.

IMPRESSION / DIAGNOSIS:

Chronic left knee pain with sudden onset, persistent symptoms exacerbated by cold exposure, possibly indicative of early degenerative joint disease or mild inflammatory arthritis, with a genetic predisposition considered.

RECOMMENDATIONS:

- Order MRI of the left knee to assess for potential structural abnormalities or early degenerative changes.
- Recommend physical therapy focusing on knee strengthening exercises.
- Consider referral to rheumatology for evaluation due to family history of autoimmune conditions.
- Advise on lifestyle modifications to manage cold exposure and potential triggers.

***** PRELIMINARY REPORT (NOT VERIFIED) *****